Improving patient experience by identifying & addressing injection training gaps via a novel clinical product: a test & learn specialty pharmacy program

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ABSTRACT

Background:
The incidence of chronic autoimmune conditions are rising significantly. Many patients require costly self-injectable, specialty medications to be administered at home. Studies have shown that only 61% of patients read the instructions for use and 84% make errors when using autoinjectors. These errors can be mitigated through proper training and onboarding.

Objectives:
This study measures patient satisfaction and addresses training injection gaps via a live, interactive, face-to-face injection training call with a specialty pharmacy clinician.

Methods:
A retrospective analysis was performed from a single-center specialty pharmacy. The criteria for inclusion involved treatment naïve and treatment-experienced patients receiving a self-administered, injectable medication. Patients received pharmacist counseling. At the professional judgment of the pharmacist, they offered the live, video chat. If the patient accepted (opted-in), their acceptance was recorded, and an appointment was made at a time convenient to the patient. Real-time feedback on injection technique was provided to the patient, including instilling confidence in injecting the medication, medication device education, and side-effect management. Motivational Interviewing was used to help the patients make sense of their condition or therapy, address knowledge gaps, and overcome barriers to reach their desired outcomes. Clinical call documentation was analyzed over a two-month time period. The analysis included answers to closed-ended yes/no questions: Was the patient trained on how to correctly administer the medication prior to this call? Would the patient recommend this training to someone else who may be starting this medication? and open-ended question: Of the patients that were trained previously, what was their goal of the call?

Results:
Of the nineteen opted-in patients, 68% of the patients were not previously trained on how to administer the medication and closed a knowledge gap through the injection training. Of the patients that were previously trained, their goals were: reassurance of injection technique, education of all resources available for their condition, confidence for their caregiver, and overcoming barriers: the fear of self-injecting the medication and anticipating painful injections because "the needle would be large since the pen is large." Patients were highly satisfied, overwhelmingly recommending this service 100% of the time answering yes to: Would the patient recommend this training to someone else who may be starting this medication?

Conclusions:
A live injection training call with a specialty pharmacy clinician delivered high patient satisfaction in patients that had not received injection training. Additional analysis would be required to determine if training call closed the knowledge gap or resolved specific patient and caregiver barriers.

BACKGROUND/SIGNIFICANCE

The prevailing model places the responsibility of device training and education on the provider and patient, assuming that device training will occur at the provider's office and that the patient will read additional instructions provided by the pharmacy prior to administering the medication.

A 2018 survey of healthcare providers found that 49% of patients prescribed self-injection devices did not receive in-office training. Similar training rates were observed for other drug delivery devices where the patient was trained in the office and required additional training once they had the medication in hand.

The addition of specialty pharmacy training is based on the hypothesis that patients oftentimes are overwhelmed while at the prescriber's office because they are processing their diagnosis, meeting a variety of healthcare providers (RNs, laboratory assistants, physicians, etc.), managing paperwork, etc. As such, when they are trained in the office for the first time, they may not fully absorb all the information and advice being shared with them despite the use of medication guides, IFUs and instructional videos.

The limitations of the current model provide the rationale for additional device training. Introducing a novel clinical product that provides a live opportunity for specialty pharmacies to perform injection training should greatly improve technique among both treatment naive and treatment experienced patients by (1) reinforcing any training previously completed (2) providing patients with an opportunity to meet individually with a clinician addressing any questions or concerns they may have (3) giving clinicians the opportunity to observe and provide feedback on the call.

OBJECTIVES

This study aimed to improve patient experience and injection technique through a novel clinical product identifying and addressing injection training gaps and patient-centric goals for training via a face-to-face call with a specialty pharmacy clinician among treatment naïve and treatment experienced patients.

METHODS

Inclusion Criteria:
- Treatment naïve and treatment-experienced patients receiving a self-administered injectable medication
- Received an initial pharmacist counseling call
- Opted-in for live, interactive injection training with a clinician

Exclusion Criteria:
- None

STEP 01
Live, interactive appointment was made with the patient and the clinician.

STEP 02
Real-time feedback was given focused on instilling confidence in injecting the medication, education on the medication device, and side-effect management.

STEP 03
Motivational Interviewing was used to help the patients make sense of their condition or therapy and reach their desired outcomes.

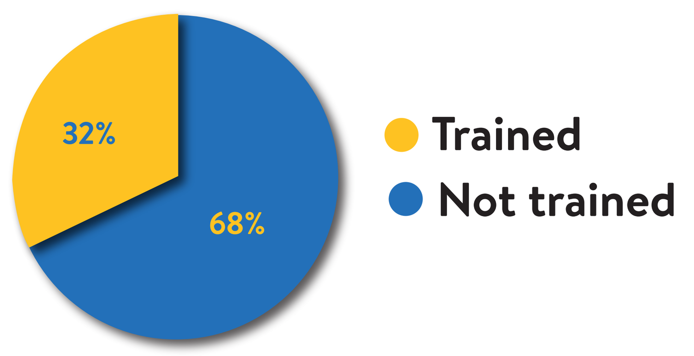
STEP 04
Clinical call documentation was analyzed over a two-month time period.

STEP05
Analysis included answers to the following questions: Was the patient trained on how to correctly administer the medication prior to this call? Of the patients that were trained, what was the goal of the call? Would the patient recommend this training to someone else starting an injectable medication?

RESULTS

- The specialty pharmacy retrospectively reviewed and analyzed documentation from live injection training calls for nineteen opted-in patients over a two-month study period.
- Of the nineteen patients, thirteen of the patients were not previously trained on how to administer the medication.

Training on administration of the medication prior to the live injection training

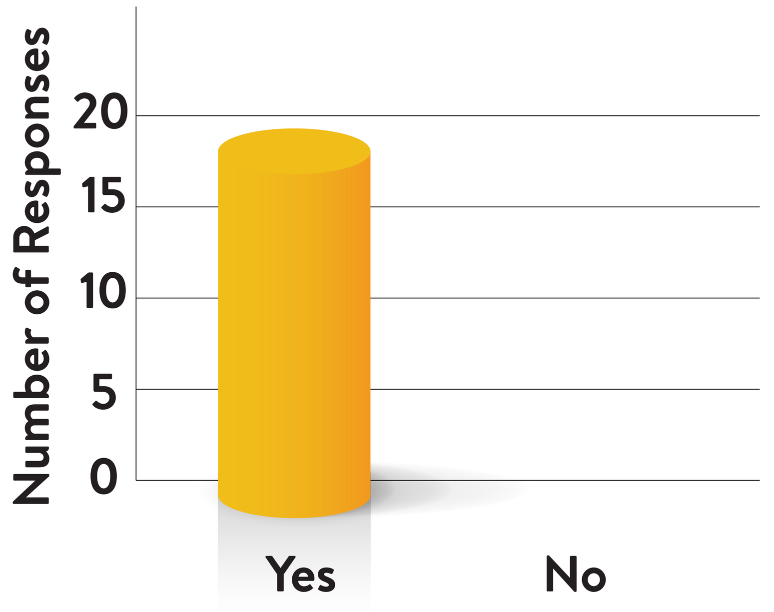

32%
68%
Trained
Not trained

Patient-centric goals achieved during the live injection training call:
Although patients were previously trained on the administration of the medication, these patients wanted reassurance they were completing the injection correctly and utilizing all resources available for their condition, and as a result, they experienced the resolution of unfavorable side effects that were a result of improper injection technique such as itching, mild rash, and pain at the injection site.

- Reassurance of injection technique
- Overcoming fear of size of injection
- Education of all resources available for condition
- Confidence for their caregiver

Patient satisfaction:
Patients were highly satisfied, overwhelmingly recommending this service 100% of the time by responding affirmatively to the question, "Would you recommend this to someone else who may be starting this medication?"

Patient Satisfaction Survey


Number of Responses
20
15
10
5
0
Yes
No

CONCLUSIONS

A live injection training call with a specialty pharmacy clinician delivered high patient satisfaction. Live injection training with patients has been associated with preventing unnecessary patient follow-up visits and calls to the provider or specialty pharmacy pertaining to administration of their medication, thus lowering healthcare costs for patients associated with these specific physician's office visits. Patients can avoid injection training technique errors with proper training prior to administration of their medication. Patient satisfaction affects clinical outcomes and improves patient-centered delivery of quality healthcare. In this retrospective analysis, treatment naïve and treatment experienced patients who had or had not previously been trained on administration of their medication, overwhelmingly recommended this product to another patient starting an injectable medication 100% of the time. Many patients wished this product was available when they first started their medication and other previous injectable treatments. Additional analysis would be required to determine if the training call closed knowledge gaps or resolved specific patient and caregiver barriers

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Authors of this poster have nothing to disclose.

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